

CH SHARIF AND SAEED HOSPITAL

Panel Patient Complete Flow v7.2

Permanent panel identity, admission, department billing, patient share, panel receivable, discharge and realized settlement.

FINAL SOURCE OF TRUTH - VERSION 7.2

Admission begins at Front Desk. Department bills remain separate. Central collections are allocated department-wise. Doctor discharge re-authentication, outsourced department settlement, Panel receivables, high-cost medicine approval, doctor-sponsored discount and doctor tax rules are included.

Supersedes v7.1 for all revised admission, billing, department settlement, doctor finance and panel-flow rules.

1. Panel Patient Identity

- Panel Patients are permanent reusable patient masters.
- Front Desk searches the Panel Patient Registry and verifies active patient/member and active panel company before creating appointment/encounter/admission.
- The visit/admission stores a snapshot of panel/member identifiers and the contract/tariff version used.

2. Panel Admission at Front Desk

Panel admission intake

1	Search Panel Patient.
2	Validate panel/member active status.
3	Resolve panel company, member ID, authorization/referral/preauthorization requirements and available coverage.
4	Select admitting doctor/department and create admission file.
5	Resolve department/service fulfillment mode and contract eligibility.
6	Collect required patient co-pay/advance if policy requires it.
7	Send admission file to Admission Portal.

3. Department Billing for Panel Patient

Department line	What system stores
Gross / standard	Reference standard amount if needed for audit
Contract/Tariff	Panel-negotiated department/service amount
Patient Share	Co-pay/non-covered portion due from patient
Panel Receivable	Covered portion due from panel company
Discount/Adjustment Source	Panel contract vs hospital-funded vs doctor-sponsored kept distinct
Paid/Realized	Patient share collected and later panel remittance realized tracked separately

4. Interim Statement

- Shows Hospital, Lab, Pharmacy, Neurology and other department subtotals.
- For each department, displays Patient Share and Panel Receivable separately.
- Shows patient advances/partial payments and current patient outstanding.
- Panel receivable is not shown as patient cash due when valid panel coverage applies.

5. Discharge and Financial Closure

Panel discharge

1	Doctor clinically discharges using credentials and summary.
2	Front Desk finalizes all department invoices and contract calculations.
3	Patient share/co-pay is cleared or handled according to approved policy.
4	Panel claim/authorization references are completed and Panel Receivable remains on panel ledger until paid.
5	Final patient-facing statement distinguishes Patient Paid/Outstanding from Panel Receivable.

6. Outsourced Provider Settlement for Panel Cases

Outsourced department settlement is based on realized money, not merely panel billing. Eligible realized amount = patient share collected for the department + panel remittance actually received for the department, subject to provider contract and configured withholding/tax.

7. High-Cost Medicine for Panel Patients

- Hospital high-cost medicine policy applies independently of payer type.
- Panel coverage does not automatically bypass attendant/management authorization.
- If panel contract requires separate preauthorization, both hospital policy and panel authorization must be satisfied before dispense.